



FOR YOUR DEVELOPER

The eight-quarter plan, and what each stage lets you say

This document is for your developer.

Status labels apply throughout: KNOWN, VERIFIED, ASSUMPTION, ESTIMATE, TO VALIDATE, PROPOSED. Nothing in this document claims traction, approvals, partners or revenue that do not exist. Governing file: FACTS_BASE.

PREPARED FOR

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VERSION

1.0 · Confidential draft

This document is for your developer.

It shows what ships each quarter from late 2026 to late 2028, what has to be true before it can ship, and what you are allowed to claim afterwards.

In one minute

- Eight quarters take MaternaLink from an unaudited prototype to a version 2 product with real evidence.
- Nothing on this plan is real until the code and name ownership is settled, in late 2026.
- Each quarter is blocked by something outside our control: a hospital site, a partner, a gateway, an ethics committee.
- What you may say is set by the evidence stage you have reached, not by what the software can do.
- Every date here is a plan, not a promise. Slippage in one gate moves everything after it.

What to do

Action	Who	By when
Settle code and name ownership, and audit the prototype	Founder	December 2026
Find the UK evaluation site and the Nigeria partner, in parallel with the build	Founder	March 2027
Treat the safety case, DPIA, DTAC form and certificates as build deliverables	Developer	March 2027
Design the version 2 flagging feature early so the device decision is made on paper	Developer	September 2027
Enforce the permitted-claims table in every deck, page and proposal	Founder	Always

The eight quarters

Quarter	What ships	What must be true first	What it lets you say
Q4 2026, Oct-Dec	Discovery with 20 women and 10 professionals per market; requirements signed off; design system; infrastructure and pipelines; build weeks 1-6 (login, tenancy, enrolment, consent, schedule, reminders, gateway adapters)	Company incorporated; code and name ownership documented; prototype audited; safety officer contracted; hazard log opened; build funded at £220k-£320k UK-led or £140k-£210k blended	Nothing about the product. Only: "we are building, informed by interviews with 20 women and 10 professionals"
Q1 2027, Jan-Mar	Build weeks 7-16: messaging and the deadline engine, rota, diary, content library (about 40 UK and 20 Nigeria items), offline sync, console, handover, escalation, defaulters, dashboard, admin and audit. Internal alpha on invented data	Each must-have feature has a written decision that it is not a medical device; the safety case is signed; the DPIA is approved; the DTAC form is filled; Cyber Essentials certified; penetration test and accessibility audit passed	"The first version is complete and entering a service evaluation." Nothing more
Q2 2027, Apr-Jun	Version 1.0 live in one UK community midwifery team. Version 1.1: WhatsApp, Yoruba content and interface, recorded voice reminders, pre-visit summary	The site has done its own safety assessment; the NHS toolkit submission is in; the Nigeria data-protection registration is done if the thresholds apply; the SMS sender ID is live	"In use in a service evaluation at [site]", with their written permission, plus plain usage numbers. No effectiveness language

Quarter	What ships	What must be true first	What it lets you say
Q3 2027, Jul–Sep	Version 1.2: USSD menus, DHIS2 export, referral notice to the receiving facility, field mode for defaulter tracing, cost tuning for SMS volume	Nigeria pilot funded at £0.1m–£0.3m; Cyber Essentials Plus certified; Nigeria ethics approval; USSD short code leased; state data-sharing agreement signed	Service evaluation results as plain descriptions, for example "median acknowledgement time X hours". "Pilot under way in [state] with [partner]", never "government partner"
Q4 2027, Oct–Dec	Version 2.0: read-only FHIR view for future hospital integration, demographics lookup, audit and reporting improvements. Rule-based flagging of self-recorded readings is designed but not released	A written decision with counsel on whether flagging and scored questionnaires make it a medical device. If yes, the Class I route is resourced. UK feasibility study has ethics approval	No change. Feasibility results do not exist yet
Q1 2028, Jan–Mar	Version 2.1: partner access with consent and safeguarding controls; mental-health questionnaires only if the device decision cleared them; Hausa, Igbo and Pidgin libraries; transport and emergency-fund tracking for Nigeria	UKCA Class I registration if it applies; safety case version 2; DPIA version 2 for partner access; toolkit resubmission; translation partners for Hausa and Igbo	Nigeria pilot interim numbers, with the partner's permission. "Feasibility study under way"
Q2 2028, Apr–Jun	Version 2.2: hospital record export interfaces where the vendor supports them, FHIR handover export, the version 3 research design, analytics hardening	Multi-site formal evaluation started; an academic partner found; data-sharing agreements and ethics applications for the research track	"Formal evaluation under way." Feasibility results once published
Q3 2028, Jul–Sep	Version 2.3: hardening, cost work, Nigeria hosting cell if a contract requires it, accessibility re-audit. Version 3 retrospective modelling, in a separate research environment under approvals	The decision on whether to build a quality system for a version 3 prediction device (24–36 months and £1.5m–£4m to a clinical claim); annual penetration test; certificate renewals	Still no prediction or outcome claims. Research is described as research

If funding is only partial in Q4 2026, build the Nigeria SMS slice first: enrolment, schedule, SMS and the defaulter list. It needs the least screen work and can earn a programme licence soonest.

The evidence stages, and what each one permits

Stage	What it is	Who governs it	What it answers	What you may say afterwards	What you may never say at this stage
Discovery, Q4 2026	Interviews, watching, designing together. No patient data	Consent for the interviews only	What women and staff need, which channels, which languages, what deadlines are normal	"Designed with input from 20 women and 10 professionals"	Anything about how the product performs
Service evaluation, Q2–Q3 2027	One site, operational measures before and after, plus a usability score	The trust's own service-evaluation governance, its safety assessment, and the DPIA	Is it usable, is it safe in operation, do staff adopt it, how fast are messages acknowledged, how many contacts are completed	"In use in a service evaluation at [site]", usability scores, plain operational numbers	Effectiveness, "improves outcomes", "validated", "proven"

Stage	What it is	Who governs it	What it answers	What you may say afterwards	What you may never say at this stage
Feasibility study, Q4 2027–Q1 2028	Two sites, mixed methods, registered in advance	Health research ethics approval, a sponsor, patient and public involvement	Is a full evaluation feasible: recruitment, retention, data completeness, acceptability	"Feasible and acceptable in a study of N women across two sites"	Efficacy, "proven", any outcome claim
Nigeria pilot, Q3 2027–Q3 2028	One or two local government areas, 500–2,000 women, compared with similar areas where possible	Nigerian ethics approval, a state data-sharing agreement, the data-protection act	Reach, engagement, contact completion against baseline, defaulter tracing time, cost per woman	"Pilot in [areas] with [partner]: reach X%, contact completion up Y points against baseline", as observed, with caveats	"Reduces maternal mortality"; "government partner"
Formal evaluation, from Q2 2028	Multi-site, registered in advance, analysed independently	Health research ethics, a sponsor, trial registration, data monitoring	Effect on process measures and on women's experience; safety events; cost	"Associated with [effect] on [process measure] in a multi-site evaluation"	Any reduction in death or serious illness, unless a study is powered for it. That is beyond this plan
Version 3 research	Retrospective modelling under data-sharing agreements, with a fairness analysis	Ethics approval in both countries; no production data without an approved data flow	Is a signal detectable, and is it fair across groups	Research findings, in publications only	Any clinical claim; any product feature

The gates that block everything

Gate	What must be true	Who owns it	When
G0 Ownership	Written assignment of code, brand and proposals; prototype audited; company name cleared	Founder	Q4 2026
G1 Classification	Intended-use statement adopted; a written decision per feature that it is not a device	Developer and counsel	Q1 2027
G2 Clinical safety	Hazard log complete; safety case report signed by the safety officer; the site does its own assessment	Safety officer	Q1 2027
G3 Data protection	DPIA approved; data-processing agreements; records of processing; ICO registration; privacy notices in her languages	Data protection officer	Q1 2027
G4 DTAC	The NHS assessment form completed across its five areas. It applies before a pilot, not only before a purchase	Developer	Q1 2027
G5 Cyber Essentials, then Plus	The five controls evidenced, then tested by an assessor	Developer	Q1 2027, then Q3 2027
G6 NHS toolkit	Supplier submission for the current cycle, as a Category 3 supplier	Developer and DPO	Q2 2027, then yearly
G7 Nigeria registration	Registered with the NDPC if the thresholds apply; a named officer; a Nigerian DPIA	Nigeria lead and counsel	Q2 2027
G8 Device decision	A written decision on flagging and scored questionnaires. If it is a device, the Class I route is funded	Developer, safety officer, counsel	Q4 2027

Gate	What must be true	Who owns it	When
G9 UKCA Class I	Technical file, self-declaration, MHRA registration, post-market plan with 15-day incident reporting	Regulatory lead	Q1 2028, if it applies
G10 Quality system	Decision on building a full quality system for a version 3 prediction device	Founder	Q3 2028

What would change this plan

Signal	Change
Discovery shows UK women overwhelmingly prefer WhatsApp to an app	Pull WhatsApp into the first build, in place of web push. Do the DPIA earlier
A northern Nigerian state partner appears first	Hausa content moves to version 1.1 and Yoruba to 1.2
Counsel judges the diary display or the handover summary to be a device	Redesign those features before go-live and add a Class I route to Q2 2027
The prototype audit is clean and ownership is assigned	Reuse some interface components; the build shortens by about two weeks
No academic partner can be found for the formal evaluation	Run a rigorous internal evaluation with an independent evaluator, and label it as exactly that

Words explained

Word	What it means
Gate	Something that must be finished before the next stage can start
Service evaluation	Watching a service already running. It is not research and proves nothing about effectiveness
Feasibility study	A small study asking whether a bigger study would work
Formal evaluation	The real study, planned and registered in advance, analysed by someone independent
Powered	A study with enough people in it to detect the effect it claims. Ours are not
DTAC	The NHS checklist a digital product must pass before a hospital can buy or pilot it
NHS toolkit (DSPT)	The annual data-security self-assessment NHS suppliers must submit
NDPC	Nigeria's data protection regulator
UKCA Class I	The lowest-risk UK medical device route, self-declared and registered with the MHRA
ESTIMATE / TO VALIDATE	Our best guess, not a fact / nobody has checked this yet